Pinched nerve in your neck? Here's what to do

by Ishaq Syed, MD
Feb 3, 2025

Have you ever experienced a sudden, sharp, burning feeling that starts in your neck and then travels down your arm? You might even start to feel a stabbing pain in your shoulder or numbness or tingling in your fingers and hand. A stiff neck or a headache might come next.

Did you sleep wrong? Did you perform an exercise incorrectly? Actually, it's likely that if you're experiencing these uncomfortable symptoms, you may have a pinched nerve.

A pinched nerve, also known as cervical radiculopathy, is a common cause of neck and arm pain. It occurs when several structures in the cervical spine press on a nerve as it branches from the spinal cord and travels down your arm, where it provides motor and sensory function.

Let's look at the causes of a pinched nerve and when you should seek treatment.

What causes a pinched nerve in neck?

Two of the most common culprits behind a pinched nerve are herniated discs and bone spurs. While rarer, a pinched nerve in the neck can also come from lifestyle and medical issues such as:

- Age-related degeneration
- Neck injury or trauma
- Poor posture

Here's what you need to know about these causes of neck and arm pain–and when to consult an orthopedic specialist for help.

Herniated disc

A disc herniation occurs when the soft gel-like material (nucleous pulposis) in the center of the disc leaks out of a defect in the stronger outer ring (nucelous fibrosis). The gel-like material can cause pressure on a nerve, resulting in pain, numbness and tingling that radiates from your neck, sometimes into your shoulder blade and down your arm.

The good news is some soft disc herniations can gradually resorb on their own with time and conservative treatment. Common treatment plans often include non-steroidal anti-inflammatory (NSAIDs) medications, physical therapy and rest.

Bone spurs

Bone spurs occur as a part of cervical spondylosis, which is a term referring to neck arthritis. This is an age-related process and a result of wear and tear. Over time, your joints and discs degenerate, which can result in overgrowth of bone–commonly referred to as bone spurs. Pain from these spurs can radiate from the joints of the vertebra as the nerve exits the spine and travels down your arm.

Bone spurs typically do not resolve on their own. However, your discomfort and any other symptoms can be treated with conservative treatment like the methods for a herniated disc: NSAIDS, medication, physical therapy and rest.

What if your neck and arm pain doesn't go away?

If you are experiencing pain, there are some immediate things you can do to alleviate the discomfort:

- Ice and heat therapy (20 minutes at a time)
- Over-the-counter pain relievers
- Gentle neck stretches
- Good posture while sitting and working
- Ergonomic workspace setup

You might also talk to your healthcare provider about other strategies:

- Physical therapy exercises
- Regular neck stretching routine
- Core and neck strengthening
- Sleeping with proper neck support
- Stress reduction techniques
- Regular movement breaks during desk work

One thing to remember is that you should always stop any activity that increases discomfort and consult a doctor if symptoms worsen or persist.

If your nerve pain does not resolve with conservative treatment or if the compression causes significant pressure on the spinal cord or weakness, a spine surgeon may recommend surgery to alleviate the pressure.

Surgical approach to a pinched nerve in neck

There are various surgical options for the treatment of a pinched nerve. The technique and approach are determined by the surgeon, taking into consideration various factors, including:

- The location of the nerve compression
- Alignment of your spine
- Determining the least invasive approach to alleviate the structural compression on the nerve

If you're dealing with persistent neck or arm pain, talk to your doctor about your options, including possible surgery. Modern advancements and minimally invasive techniques have made surgery safer, led to shorter hospital stays and made it possible for a rapid return to normal activity and work.

About the Author

Ishaq Y. Syed, MD, is an orthopedic surgeon on the medical staff at Baylor University Medical Center – Dallas. He specializes in the treatment of spinal conditions including arthritis of the spine, degenerative disc disease, herniated discs, radiculopathy (pinched nerves and sciatica), spinal deformities, traumatic injury to the spine, spinal stenosis and spine tumors.